

Complete Evidence-Based Model Answers • Q1 + Q2 + Q3 • with Active-Recall Pack
Proctored Online Examination • January Semester 2026

SUSS • SWK501 HUMAN LIFESPAN & SOCIAL ECOLOGY

The Complete January 2026 POE Model Answers & Active-Recall Pack

Q1 (55 marks) — early-childhood development: biopsychosocial needs of two- and four-year-olds, emotion coaching vs dismissing, and a dual-focus parent intervention

Q2 (20 marks) — Marcia's identity-status theory (with emerging adulthood and neo-Eriksonian process models) for adolescent identity work

Q3 (25 marks) — cognitive ageing and SOC across Singapore's active-ageing ecosystem (Healthier SG, Healthy 365, ActiveSG, AIC, AACs)

One exam-ready compendium: full-distinction model answers with APA citations, rapid-revision matrices, and a printable flashcard + quiz pack — grounded in longitudinal, meta-analytic and official Singapore evidence.

ORIENTATION

How to use this compendium

This document compiles evidence-based model answers to the three compulsory questions of the SWK501 **January 2026** Proctored Online Examination (Q1 = 55 marks; Q2 = 20 marks; Q3 = 25 marks; 100 total), followed by an active-recall pack. Each answer keeps the SUSS marking hallmarks: multidimensional, non-deterministic reasoning; explicit theory with named authors; and Singapore-grounded application.

***Which paper is which (to avoid cross-semester confusion):** this January 2026 paper's Q3 is **cognitive ageing + SOC**. That differs from July 2025 (empty nest) and January 2025 (the Max delinquency case). Its Q1 is **early childhood** (a two- and four-year-old), not the young-adult Tan case of July 2025.*

- **Section 1** answers Q1 (biopsychosocial needs; emotion coaching vs dismissing; dual-focus intervention) using Erikson, Piaget, **Vygotsky**, **Gottman**, attachment, **temperament** and Bronfenbrenner.
- **Section 2** answers Q2 (Marcia's four statuses) and layers on **emerging adulthood** and neo-Eriksonian **process models**.
- **Section 3** answers Q3 (cognitive ageing + SOC) and compares Singapore's **Healthier SG / Healthy 365 / ActiveSG / AAC** ecosystem, proposing a practical integration.
- **Sections 4–6:** rapid revision; flashcards + MCQ quiz with answer key; APA references.

KEY TAKEAWAY — Development at every age is multidirectional and transactional: behaviour is the product of the child/person, biology, relationships, institutions and culture interacting over time — never a single cause. Milestones and stages are heuristics, not diagnoses.

QUESTION 1 • 55 MARKS

Early-Childhood Development & Parent Psychoeducation

The question asks for the biopsychosocial needs of a two- and a four-year-old, an appraisal of **emotion coaching** versus **emotion dismissing**, and a theoretically grounded **dual-focus** intervention for their parents. The behaviours described are not automatically misconduct; their meaning depends on age, development, temperament, family interaction and context.

1. Biopsychosocial needs of the two- and four-year-olds

Domain	Two-year-old	Four-year-old	Psychoeducational implication
Biological / motor	Rapid gains in running, climbing, self-feeding; safety awareness and impulse control still immature	Better balance, ball and fine-motor skills, dressing; wider exploration	Provide safe movement and mastery; childproof; don't expect prolonged stillness
Eriksonian need	Autonomy vs shame & doubt — wants control over eating, dressing, movement	Initiative vs guilt — initiates games, questions, plans, roles	Offer limited choices and self-help; support planning/pretend while teaching limits (Erikson)
Cognition (Piaget)	Moving from sensorimotor into preoperational; thought tied to perception	Firmly preoperational; symbolic play, intuitive, often egocentric	Explain concretely, demonstrate, use pictures/routines; don't expect adult logic
Language	Combines 2+ words; understands more than can say; frustration → crying/grabbing	Longer sentences, recounts events, asks constant questions	Expand words; for the 4yo, listen and teach turn-taking rather than silencing
Emotional	Largely co-regulated by adults; needs calm containment and labels	More words/strategies but still needs help to wait, negotiate, repair	Adult self-regulates first; secure attachment supports regulation
Social / play	Parallel play; possessiveness and toy conflict are normal	Pretend roles, negotiation, peer interest; rules still developing	Model short turns for the 2yo; coach entry, negotiation and repair for the 4yo

Theory augmentation (beyond the core). The four-year-old's "non-stop talking" is not merely Piagetian egocentric speech: **Vygotsky** reframed it as **private speech** — outer self-guidance that later goes underground as inner verbal thought, developing within the **zone of proximal development** through adult **scaffolding**. **Thomas and Chess's** temperament model adds **goodness of fit**: outcomes depend on the match between the child's temperament and caregiving demands, not the temperament alone. Both reinforce a non-deterministic, relational reading.

Reframing the behaviours. The "rebellious" two-year-old is expressing immature **will** ("I want to do it," "I can't yet say what I need"); the response is *supported autonomy* — two acceptable choices, safe self-help, transition warnings, firm safety limits. Observational longitudinal research links persistent **overcontrolling** parenting at age two with poorer self-regulation and inhibitory control at five, so the aim is to avoid intrusive over-control, not every limit. A useful script: "You want to keep playing. Stopping is hard. It is bath time — you may carry the red toy or the blue toy."

2. Emotion coaching vs emotion dismissing

Emotion coaching (from Gottman's meta-emotion research) is a pattern in which the parent notices the child's emotion, treats it as a chance to connect and teach, listens empathically, helps label the feeling, **keeps behavioural limits** and guides problem-solving. A practical five-step sequence: (1) notice the emotion and regulate the adult's own response; (2) acknowledge the experience; (3) listen and validate the feeling without agreeing to every demand; (4) name the emotion; (5) set limits and problem-solve. Validation concerns the *feeling*, not every action — "You are angry" is compatible with "I will not let you hit."

Emotion dismissing repeatedly minimises, ridicules, punishes, ignores or distracts from negative emotion ("Stop crying, there's nothing to be upset about") without helping the child understand or regulate it. It is often driven by parental anxiety, time pressure or difficulty tolerating distress — not lack of love.

Trigger	Emotion-dismissing response	Emotion-coaching response
2yo screams when a dangerous object is removed	"Stop being naughty. Nothing to cry about." (no language or strategy; adds shame)	"You're angry — you wanted the scissors. Scissors can hurt, so I'll keep them. You can tear this paper or use the crayons."
4yo talks over a sibling from excitement	"You never stop talking. Be quiet." (suppresses initiative)	"You're excited and want us to hear your story. Hold it in your head while your sister finishes — then it's your turn."

- **Evidence for coaching:** a meta-analysis of randomised trials (children ~18 months–7 years) found positive effects on emotion-socialisation practices, child emotional competence and behavioural adjustment; the *Tuning in to Kids* community trial increased coaching and reduced dismissing, with parent- and teacher-reported reductions in child behaviour problems.
- **Consequences of repeated dismissing:** longitudinal family-profile research links dismissive/punitive patterns with later internalising and externalising symptoms — but causality is bidirectional (intense child emotions also evoke parental dismissing), so avoid deterministic single-incident claims.
- **Critical caveat:** coaching is not unlimited expression or the absence of discipline, and not a rigid script; temperament, culture, language level and situation matter, and safety or de-escalation may come first.

KEY TAKEAWAY — Emotion coaching = empathy + labelling + limits + problem-solving (Gottman). Validate the feeling, bound the behaviour. Repeated dismissing (not one lapse) is the concern, and effects run in both directions.

3. Dual-focus multidimensional intervention plan

The **dual-focus** principle means working simultaneously with the people involved **and** the environments that sustain or reduce difficulty (biophysical, psychological, social-systemic). A suitable programme is an **eight-session FSC Parent–Child Regulation & Development Programme**, with individual assessment and preschool/childcare collaboration on consent.

Focus	Person-focused component	Environment-focused component
Assessment	Screen each parent's stress, beliefs and triggers; the child's communication, play, sleep, hearing, health, temperament and behaviour across settings	Map caregivers, childcare, work schedules, housing, finances and support (person-in-environment; bioecological)
Developmental reframing	Teach the 2yo-autonomy vs 4yo-initiative distinction; practise realistic expectations	Align expectations among parents, grandparents, helpers and preschool staff
Emotion coaching	Teach awareness, validation, labelling, limits and problem-solving via video and role-play	Shared feelings-language at home and childcare; visual emotion cards in common areas
Positive behaviour support	Labelled praise, brief instructions, limited choices, transition warnings, logical consequences	Alter antecedents: visual schedules, safe play zones, predictable mealtimes, less waiting
Play & language	Coach 10 minutes daily child-led play, conversational expansion, turn-taking	Link to libraries, playgroups, parks; collaborate with preschool teachers
Biological regulation	Establish realistic sleep/meal/calming routines; spot sensory or health concerns	Reduce evening overstimulation; coordinate routines; refer for medical/developmental review if indicated

- **Why it works (theory):** Bronfenbrenner's bioecological model locates change in repeated, reciprocal **proximal processes** — so the programme changes daily parent–child *interactions*, not just parental knowledge; it also strengthens the home–childcare **mesosystem** and addresses **exosystem** work schedules. Social-learning principles inform modelling and praise; attachment informs co-regulation.
- **Evidence:** parent-management training and Parent–Child Interaction Therapy reduce disruptive behaviour (a 25-RCT meta-analysis found moderate PMT and larger PCIT effects versus waitlist — strongest for clinical-level disruption, so do not assume those sizes for ordinary developmental concerns). Sleep should be assessed but not used as a simplistic explanation.
- **Referral criteria:** loss of acquired skills, marked language delay, limited social responsiveness, persistent hearing concerns, severe sleep disturbance, unusually intense aggression, or impairment across settings — milestone checklists prompt referral but are not diagnostic.

KEY TAKEAWAY — Dual focus = change both the parent's skills (coaching, praise, routines) and the environment (antecedents, home–childcare alignment, work/sleep conditions). Success is safer behaviour, quicker recovery and less family distress — not the disappearance of age-typical autonomy or talk.

QUESTION 2 • 20 MARKS

Marcia's Identity-Status Theory (with Emerging Adulthood & Process Models)

The question asks how Marcia's Identity Status Theory can guide social work toward healthy adolescent identity development, with an example. Erikson framed adolescence as integrating earlier identifications into a coherent self, supported by a **psychosocial moratorium**; Marcia operationalised this through two dimensions.

Status	Exploration	Commitment	Likely presentation	Social-work task
Diffusion	Low	Low	Little direction; avoidance or overwhelm	Build safety, motivation, hope; small exploration opportunities
Foreclosure	Low	High	Commitment adopted from others without examining alternatives	Respect commitments; create permission for reflective choice
Moratorium	High	Low	Active questioning and uncertainty; may include anxiety	Normalise exploration; contain distress; structure it
Achievement	High	High	Commitments experienced as personally chosen	Enact commitments flexibly; prepare for future revision

- **How it helps:** it replaces moral judgement with developmental formulation (an uncertain teen may be in moratorium; a compliant one may be foreclosed), directs **multi-domain** assessment (career, values, ethnicity, relationships, spirituality — status can differ by domain), and **matches intervention to process** (exposure and experiments for diffusion; reflective questions for foreclosure; structured exploration for moratorium; realistic planning for achievement).
- **Evidence for a dynamic, non-linear view:** a meta-analysis of 124 studies found more progressive than regressive change but that many young people had not reached achievement by young adulthood; a five-wave study (ages 12–20) found both stability and deepening exploration — identity work is ongoing, not a single irreversible transition.

***Emerging adulthood + process models (propose these).** Arnett's **emerging adulthood** reframes the late teens–twenties as an extended identity-exploration window — useful because much identity work now continues past adolescence. Neo-Eriksonian **process models** refine Marcia's static statuses: Luyckx et al.'s **dual-cycle** separates exploration in breadth/depth from commitment making/identification; Crocetti and Meeus's **three-factor** model adds **reconsideration of commitment**; Berzonsky's **identity styles** (informational, normative, diffuse-avoidant) add a social-cognitive processing lens; Phinney's **ethnic-identity** and McAdams's **narrative identity** widen the content to culture and life-story.*

Illustrative case (Mei, 16). Mei says she will be a doctor because her parents planned it, performs well, but becomes anxious about hospitals and privately loves design. This suggests **foreclosure** in the vocational domain — not a diagnosis or a judgement that medicine is wrong. Intervention: validate family hopes; values/strengths assessment; informational interviews (doctor, allied-health, social worker, designer); a short design project and health-sector volunteering; a family meeting reframing exploration as responsible decision-making. Healthy development is shown by increasing ownership and coherence, not by steering her to one occupation.

- **Culturally responsive practice:** ask whether apparent foreclosure reflects coercion, valued interdependence, or both; Western assumptions that autonomy requires rejecting collective expectations are inappropriate, and ethnic-identity research shows exploration and commitment can respond to salience and discrimination.
- **Critical appraisal:** the four categories can oversimplify continuous, domain-specific processes; original samples were largely Western students; “achievement” is not a permanent superior endpoint; and the model says little about structural constraints or the ethics/content of commitments.

KEY TAKEAWAY — Use Marcia as a flexible map for dialogue, not a label: promote safe exploration, personally meaningful commitment, family dialogue and real opportunities — and enrich it with emerging-adulthood and process models rather than treating status as fixed.

QUESTION 3 • 25 MARKS

Cognitive Ageing & SOC across Singapore's Active-Ageing Ecosystem

The question requires an evaluation of cognitive development in late adulthood and locally grounded proposals showing how SOC contributes to successful ageing. Late adulthood is **not** uniform decline: abilities differ in trajectory and between-person variation grows with age.

1. Cognitive development in late adulthood

- **Mechanics (fluid):** processing speed, working memory, inhibition, divided attention and episodic recall tend to become less efficient on average.
- **Pragmatics (crystallised):** vocabulary, general knowledge, expertise and practised procedures are comparatively resilient and may rise into the 60s–70s — e.g., a 72-year-old retired nurse learns a new app slowly but reads a health warning sign faster.
- **Memory & executive function:** free recall and dual-tasking are more vulnerable; recognition, semantic and procedural memory are stronger; older adults adapt via prioritising, routines and reminders (metacognitive management, not failure).
- **Heterogeneity & pathology:** Newcastle 85+ evidence shows many very old adults decline little over five years — normal ageing is not dementia. Loss of familiar skills, getting lost, or functional decline warrants assessment; sleep, sensory loss, medication, depression and isolation can mimic “old age.”
- **Modifiable influences (cautiously):** education predicts higher level but may not slow decline (Victoria Longitudinal Study); aerobic exercise has a *small* average effect on episodic memory (55+), clearer at 55–68; loneliness is modestly associated with poorer cognition — none proves reversal or dementia prevention.

2. SOC and how it contributes to successful ageing

Baltes and Baltes' SOC explains how people maintain valued functioning amid change. **Selection** (elective or loss-based) prioritises meaningful goals; **Optimisation** invests and practises resources; **Compensation** adopts alternative means. They are interdependent: selection without optimisation is mere intention; optimisation without compensation fails after loss; compensation without a meaningful goal preserves activity but not purpose. In the Berlin Aging Study, greater SOC use was associated with higher subjective well-being and lower loneliness (correlational, so association not proof).

3. Singapore's active-ageing ecosystem — compare & contrast

Initiative (lead)	Primary lever	Key strength	Key limitation
Healthier SG (MOH)	Clinical prevention: family-doctor enrolment, personalised Health Plan, screening, social prescribing	Continuity of care; medical anchor; can refer into community (1.3M+ enrolled by Dec 2025)	Voluntary; GP workload; medically framed; social needs secondary
Healthy 365 (HPB)	Behavioural self-tracking + gamified rewards (steps, MVPA, sleep); senior modules Age Strong, Steady Lah, Live Well Age Well	Scalable, low-cost, habit-forming, measurable; seated/senior options	Digital divide; tracker faults; engagement decay; self-report
ActiveSG / Active Health (SportSG)	Physical fitness: heartland gyms, Active Health Coaches, functional assessments	Accessible facilities; professional coaching; functional focus	Requires mobility/motivation to attend; limited reach to frail/homebound
AACs / Age Well SG (MOH)	Social connectedness + active ageing in the community; befriending, activities, screening	Proximity (230+ centres; >80% of seniors near a touchpoint); catches isolation	Variable service mix/quality; outreach-dependent; not clinical
AIC (statutory coordinator)	Care integration: connects health + social, referrals, provider support	The connective tissue enabling social prescribing across the system	Coordination gaps; not a direct service to the senior

*Mapping the ecosystem to SOC: the Healthier SG Health Plan supports **selection** (goals) and loss-based revision; Healthy 365 and Active Health drive **optimisation** (practice, MVPA, coaching); seated options, HPB trackers, AAC buddies, transport and age-friendly design (e.g., Green Man+) provide **compensation**; and AACs anchor connectedness. Age Well SG (\$3.5b over the decade; ~\$1.9b to MOH) funds the community backbone.*

4. A practical way forward: an AIC-coordinated, goal-anchored pathway

- **Selection first:** at the Healthier SG Health Plan consult, the family doctor and older adult set *one meaningful life goal* (not only clinical targets) — e.g., “keep leading my line-dance group.”
- **Social-prescribe the match:** refer (via AIC / the regional health cluster) into the lever that fits — Healthy 365 *Steady Lah/Age Strong* for daily practice, ActiveSG *Active Health* for supervised training, and the local AAC for social connection and screening.
- **Design compensation in:** seated exercise options, loaned trackers, digital-literacy help and buddies at the AAC, and transport support — so the digital divide and mobility/affordability barriers don't exclude the frailer or poorer, keeping SOC from being individualised.
- **Close the loop:** AIC and the AAC feed participation and function back to the Healthier SG GP, so the plan is reviewed and adjusted — turning four parallel programmes into one SOC-aligned journey.

Proposed AAC activity (SOC-structured): an eight-week *My Neighbourhood Digital Story-Walk* — each senior selects one meaningful story goal and mode (photo walk, indoor route, or seated project); optimises through weekly practice of pacing, photography/audio, sequencing and rehearsal; and compensates with large-print cards, voice-to-text, buddies, templates and transport. Success is a meaningful story completed *with* support, not unaided independence.

- **Critical evaluation:** SOC evidence is largely correlational/self-report; healthier, better-resourced people may both use SOC and fare better. “Selection” can mask exclusion (staying home because transport is inaccessible); compensation may be unaffordable; and the digital layer risks widening inequality. SOC contributes to successful ageing *only* when individual strategy is backed by accessible care, transport, technology, income and community — exactly what an integrated, equity-designed ecosystem must provide.

KEY TAKEAWAY — Late-adulthood cognition is multidirectional (mechanics decline, pragmatics resilient) and dementia is not normal ageing. SOC gives the person-level strategy; Singapore's power is the ecosystem — Healthier SG (select), Healthy 365 + Active Health (optimise), AAC + design (compensate), AIC (integrate). Build compensation in so SOC isn't individualised.

SECTION 4 • RAPID REVISION

One-screen recall across the paper

Q1 — Early childhood

- 2yo = **Autonomy vs shame/doubt**; 4yo = **Initiative vs guilt** (Erikson). Both preoperational (Piaget). 4yo talk = Vygotsky **private speech**, not just egocentric.
- **Emotion coaching** (Gottman): notice → acknowledge → validate → label → limit + problem-solve. Validate feeling, bound behaviour. Dismissing (repeated) → worse outcomes; effects bidirectional.
- **Dual focus**: change parent skills AND environment (antecedents, home–childcare mesosystem, sleep/work). Bronfenbrenner proximal processes; PMT/PCIT evidence; assess-don't-assume; clear referral criteria.

Q2 — Identity

- Marcia = exploration × commitment → diffusion / foreclosure / moratorium / achievement; match intervention to status; multi-domain; non-judgemental.
- Layer on Arnett (emerging adulthood) + process models (Luyckx dual-cycle; Crocetti–Meeus three-factor + reconsideration; Berzonsky styles; Phinney ethnic; McAdams narrative). Achievement is not permanent.

Q3 — Cognitive ageing + SOC

- Mechanics (fluid) decline; pragmatics (crystallised) resilient; huge variability; dementia ≠ normal ageing. SOC = Selection/Optimisation/Compensation (Berlin Aging Study).
- Ecosystem: Healthier SG (select/clinical) + Healthy 365 & Active Health (optimise) + AAC/Age Well SG (connect/compensate) + AIC (integrate). Practical way forward = goal-anchored, social-prescribed, equity-designed pathway.

SECTION 5 • ACTIVE RECALL

Flashcards — cover the right column, recall aloud, then check

Q1 — Early childhood

CUE (recall aloud)	MODEL RECALL (check yourself)
Erikson's crisis for the 2yo and the 4yo?	2yo: Autonomy vs Shame & Doubt; 4yo: Initiative vs Guilt.
How does Vygotsky reframe the 4yo's constant talk?	As private speech — self-guiding outer speech that becomes inner thought; develops in the ZPD via adult scaffolding (not merely egocentric).
Name Gottman's emotion-coaching sequence.	Notice + self-regulate → acknowledge → listen/validate → label the emotion → set limits + problem-solve. Validate the feeling, bound the behaviour.
Two-year-old “rebellion” — the right response?	Supported autonomy: two acceptable choices, safe self-help, transition warnings, firm safety limits; avoid persistent over-control.
What is “goodness of fit” (Thomas & Chess)?	Outcomes depend on the match between a child's temperament and caregiving/environment demands, not temperament alone.
What makes an intervention “dual-focus” here?	It changes parent skills AND the environment (antecedents, home–childcare mesosystem, sleep/work), altering daily proximal processes (Bronfenbrenner).

Q2 — Identity

CUE (recall aloud)	MODEL RECALL (check yourself)
Marcia's two dimensions and four statuses?	Exploration × commitment → diffusion (low/low), foreclosure (low/high), moratorium (high/low), achievement (high/high).
A compliant teen committed to a parent-chosen path is in which status?	Foreclosure — high commitment, low exploration; respect it while creating permission for reflective choice.
Which theories extend Marcia (propose)?	Arnett emerging adulthood; Luyckx dual-cycle; Crocetti–Meeus three-factor (adds reconsideration); Berzonsky identity styles; Phinney ethnic; McAdams narrative.
One key limitation of Marcia's model?	Four categories oversimplify continuous, domain-specific processes; achievement is not a permanent superior endpoint; Western/student samples.

Q3 — Cognitive ageing + SOC

CUE (recall aloud)	MODEL RECALL (check yourself)
Mechanics vs pragmatics in late adulthood?	Mechanics (fluid: speed, working memory) decline on average; pragmatics (crystallised: vocabulary, expertise) are resilient; variability is large.
Define S, O, C.	Selection (elective/loss-based goal prioritising); Optimisation (practise/invest resources); Compensation (alternative means when old ones fail).
How does Healthier SG link a patient to community activity?	Social prescribing — the family doctor refers into community partners (AACs, HPB Healthy 365, SportSG).
Map the ecosystem to SOC.	Healthier SG Health Plan = selection; Healthy 365 + Active Health = optimisation; seated options/trackers/buddies/transport = compensation; AAC = connectedness; AIC = integration.
Biggest SOC caveat for the exam?	Evidence is correlational; don't individualise ageing — accessible care, transport, technology, income and community determine whether S/O/C are feasible.

SECTION 5 (CONT.) • SELF-TEST

16-item MCQ quiz — answers on the next page

Q1. Erikson's psychosocial crisis for a two-year-old is:

- (a) Trust vs Mistrust
- (b) Autonomy vs Shame & Doubt
- (c) Initiative vs Guilt
- (d) Industry vs Inferiority

Q2. Erikson's crisis for a four-year-old is:

- (a) Autonomy vs Shame & Doubt
- (b) Initiative vs Guilt
- (c) Identity vs Role Confusion
- (d) Intimacy vs Isolation

Q3. A four-year-old's continuous talk is best explained (beyond egocentric speech) by:

- (a) Vygotsky's private speech
- (b) Freud's latency
- (c) Pavlovian conditioning
- (d) Piaget's formal operations

Q4. In Gottman's emotion coaching, validation is always paired with:

- (a) rewards
- (b) behavioural limits
- (c) time-out only
- (d) ignoring

Q5. Piaget's cognitive stage for a four-year-old is:

- (a) sensorimotor
- (b) preoperational
- (c) concrete operational
- (d) formal operational

Q6. "Goodness of fit" (Thomas & Chess) refers to the match between:

- (a) two parents' styles
- (b) child temperament and environment/caregiving
- (c) IQ and school
- (d) siblings

Q7. Changing daily parent–child interactions in a dual-focus plan targets Bronfenbrenner's:

- (a) macrosystem
- (b) proximal processes
- (c) chronosystem
- (d) exosystem only

Q8. Marcia's two identity dimensions are:

- (a) trust and autonomy
- (b) exploration and commitment
- (c) fluid and crystallised
- (d) selection and optimisation

Q9. High commitment with low exploration is:

- (a) diffusion
- (b) foreclosure
- (c) moratorium
- (d) achievement

Q10. High exploration with low commitment is:

- (a) diffusion
- (b) foreclosure

- (c) moratorium
- (d) achievement

Q11. Berzonsky's identity styles add which kind of lens to Marcia?

- (a) psychodynamic
- (b) social-cognitive processing
- (c) behaviourist
- (d) biological

Q12. In late adulthood, which typically remains most resilient?

- (a) processing speed
- (b) working memory
- (c) crystallised knowledge/vocabulary
- (d) divided attention

Q13. Using a hearing aid to keep participating is an example of SOC:

- (a) selection
- (b) optimisation
- (c) compensation
- (d) diffusion

Q14. Healthier SG's mechanism for linking patients to community activity is called:

- (a) triage
- (b) social prescribing
- (c) means-testing
- (d) step-down care

Q15. Which agency chiefly integrates/coordinates Singapore's aged-care ecosystem?

- (a) HPB
- (b) SportSG
- (c) AIC
- (d) CPF Board

Q16. The safest overall claim about cognitive ageing is that it:

- (a) is uniform decline
- (b) equals early dementia
- (c) is multidirectional with large variability
- (d) does not occur

SECTION 5 (CONT.) • ANSWER KEY

Quiz answers with one-line rationale

Q	Ans	Why
1	b	Toddlerhood centres on autonomy vs shame & doubt.
2	b	The preschool crisis is initiative vs guilt.
3	a	Vygotsky's private speech is self-guiding, not merely egocentric.
4	b	Coaching pairs validation of feeling with firm behavioural limits.
5	b	Four-year-olds are in the preoperational stage.
6	b	Goodness of fit = temperament–environment match.
7	b	Proximal processes are repeated daily interactions.
8	b	Exploration and commitment define the four statuses.
9	b	Foreclosure = committed without exploring.
10	c	Moratorium = exploring without settled commitment.
11	b	Berzonsky adds a social-cognitive processing lens.
12	c	Crystallised knowledge is comparatively preserved.
13	c	Compensation = alternative means for a goal.
14	b	Family doctors social-prescribe into community partners.
15	c	The Agency for Integrated Care coordinates aged care.
16	c	Ageing cognition is multidirectional with wide variability.

Scoring: 14–16 = exam-ready; 10–13 = solid, revisit weak items; ≤ 9 = re-read the relevant model-answer section before re-testing. Retrieval practice beats re-reading — cover the flashcard answers and say them aloud.

SECTION 6 • SOURCES

Selected references (APA; paraphrased in text)

Early childhood (Q1)

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Identity (Q2)

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Cognitive ageing, SOC & Singapore ecosystem (Q3)

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- Ministry of Health / MND / MOT. Age Well SG (S\$3.5b over the decade; ~S\$1.9b MOH); Active Ageing Centres (230+; >80% of seniors near a touchpoint). 2023 Action Plan for Successful Ageing.
- Health Promotion Board. Healthy 365 app; National Steps Challenge; Age Strong; Steady Lah; Live Well, Age Well.
- Sport Singapore. ActiveSG; Active Health (coaches, functional assessment). Agency for Integrated Care (AIC) — aged-care coordination and community referrals.

Study aid for revision. Empirical associations are population-level and probabilistic; apply them as contributing mechanisms, not deterministic rules for any individual. Verify current figures (enrolment numbers, AAC counts, budgets, programme names) against official MOH / HPB / SportSG / AIC / Age Well SG publications before submission, as these are updated periodically.